

ROI models can feel like black boxes—numbers that sound good but don't explain themselves. This assessment is different. Every value traces back to your inputs, industry benchmarks, and assumptions you can inspect. We're not selling you on a number. We're giving you a model you can stress-test, adjust, and defend internally.

You selected **3 value drivers**: Overtime Reduction, Nurse Retention, Falls Prevention. Each section walks through the logic step by step—what we're measuring, why it matters, and exactly how we calculated it.

+\$277K
NET ANNUAL GAIN
\$637K value - \$360K cost

1.8x
RETURN ON INVESTMENT
Every \$1 returns \$1.77

\$461
PER STAFFED BED
Net annual benefit each

116,800
HOURS RETURNED
Documentation time saved

WHERE THE VALUE COMES FROM

LABOR & EFFICIENCY
100% of total value
\$637K

Overtime Reduction\$434K

Nurse Retention\$203K

QUALITY & SAFETY
100% of total value
\$46K

Falls Prevention\$46K

* Shown as "Potential Value"



INVESTMENT DETAILS

YOUR CONFIGURATION

Setting
staffed beds
Price

Nursing
600
\$150/staffed bed/month

Annual Investment**\$360K**

MULTI-YEAR PROJECTION

	Year 1	Year 2	Year 3	3-Yr Total
Value	\$382K	\$541K	\$637K	\$1.56M
Cost	\$360K	\$371K	\$382K	\$1.11M
Net	\$22K	\$171K	\$255K	\$448K

Assumes 10% annual value growth with increased adoption

At **\$461 per staffed bed** in net annual value, scaling from 600 to 1800 staffed beds would increase annual benefit from \$277K to approximately \$831K. The methodology section explains how these projections work—and where your situation might differ.

Overtime Reduction

\$434K

THE THEORY

Every nursing unit has a rhythm: shift start, rounds, assessments, medications, documentation, handoff, shift end. In theory, documentation fits within the shift. In practice, it doesn't.

Nurses routinely stay 30-60 minutes past their scheduled shift to finish charting. Multiply that across your FTEs, 5 days a week, 50 weeks a year—and you're looking at tens of thousands of overtime hours. At 1.5× pay, that's real budget impact.

But here's the thing: not all overtime is documentation. Some is patient acuity. Some is understaffing. Some is just the nature of nursing. We estimate ~33% of overtime is specifically end-of-shift charting catch-up—the kind that real-time ambient documentation can address.

This is the most measurable driver in nursing. You can track it in payroll data, week over week, unit by unit. If overtime drops after Abridge deployment, you'll see it.

YOUR CALCULATION

STEP 1: DAILY OT PER NURSE

How much overtime does documentation drive?

15

min/shift =

15 min overtime

Documentation catch-up typically adds 15-30 minutes per shift.

STEP 2: ANNUAL OT HOURS

What's the total overtime impact?

900

nurses x

15

min x

260

shifts =

58,500 hrs

260 shifts = 5 shifts/week x 52 weeks average per nurse.

STEP 3: OT COST ELIMINATED

What's the savings?

58,500

hours x

\$60

OT rate =

\$434K

OT rate = 1.5× base (\$40-50/hr ' \$60-75/hr).

BENCHMARK: NURSING OVERTIME RATES

Typical OT Rate	1.5x base
Avg Hourly Rate (RN)	\$35 - \$50
OT Hourly Cost	\$52 - \$75
Doc-Related OT	30 - 45 min/shift

Documentation is a leading driver of nursing overtime.

Annual Overtime Reduction Value

0 OT hrs x \$50 x 1.5

\$434K

WHAT THIS MEANS

58,500 overtime hours eliminated annually—that's \$434K in direct payroll savings. This shows up immediately in your labor reports and is one of the most measurable impacts of ambient documentation.

Nurse Retention

\$203K

THE THEORY

Nursing turnover runs 18-25% annually—significantly higher than most hospital roles. The cost per departure is \$40-60K when you factor in recruiting, hiring, onboarding, training, and the productivity ramp for new nurses learning unit workflows.

But the real cost is harder to measure: institutional knowledge lost, team dynamics disrupted, patient relationships broken, and the burden on remaining staff who pick up extra shifts.

Why do nurses leave? The research is consistent: workload and patient ratios, schedules and work-life balance, compensation and career growth, and administrative burden and documentation.

Documentation burden shows up in every nursing satisfaction survey. It's the thing nurses didn't sign up for. They became nurses to care for patients, not to chart about caring for patients.

Here's where we're careful: nursing burnout is multifactorial. Documentation is ONE driver, not THE driver. Unlike physicians—where 50% of burnout is documentation-related—nurses face additional pressures (ratios, acuity, physical demands) that Abridge doesn't address.

We use 15% attribution for nursing (vs 30% for physicians) because we want to be honest about what documentation improvement can and can't do. It helps. It's not a silver bullet.

YOUR CALCULATION

STEP 1: ANNUAL TURNOVER

How many nurses leave annually?

900

nurses x

20%

turnover =

180.0 departures

Nursing turnover averages 18-25%.

STEP 2: BURNOUT-RELATED

How many are burnout-driven?

180.0

departures x

40%

burnout =

72.0 burnout

Documentation is a top burnout driver in nursing.

STEP 3: ABRIDGE IMPACT

How many can be retained?

72.0

at-risk x

25%

impact =

18.0 retained

Conservative estimate—full impact takes 12+ months.

STEP 4: COST SAVINGS

What's the value?

18.0

retained x

\$50K

cost =

\$203K

Replacement cost: \$40K-\$60K including recruiting/onboarding.

BENCHMARK: NURSING TURNOVER COSTS

Recruiting & Hiring	\$15K - \$25K
Onboarding & Training	\$10K - \$20K
Lost Productivity	\$20K - \$35K
Total Replacement	\$45K - \$80K

Average RN replacement cost is 0.5-1x annual salary.

Annual Nurse Retention Value

18.00 retained x \$50K

\$203K

WHAT THIS MEANS

Retaining 18.0 additional nurses per year saves \$203K in replacement costs. This is a 12+ month impact—but satisfaction and engagement improvements often appear within months.

Falls Prevention

\$46K

THE THEORY

Patient falls cost \$3K-\$30K per incident. Prevention depends on real-time risk awareness and timely interventions—both of which require time at the bedside.

Falls happen when: risk assessments are incomplete or delayed, mobility and toileting schedules aren't followed, environmental hazards aren't identified and addressed, and nursing staff are stretched too thin to provide adequate supervision.

Real-time documentation means nurses spend less time charting and more time at the bedside where they can observe early signs of fall risk—confusion, restlessness, attempts to get out of bed.

Like HAPI prevention, this is an indirect benefit. The chain is: less charting time ' more bedside presence ' better situational awareness ' faster intervention ' fewer falls. The mechanism is clear, but direct attribution is complex.

We show this as "POTENTIAL VALUE" because the causal link, while well-supported, is indirect. Your quality team can help you assess whether the assumed prevention rate fits your patient population.

Potential Value Note

This benefit has an indirect causal chain. The mechanism is well-supported in literature, but direct attribution is complex. We include this as potential value for transparency.

WHY THIS IS POTENTIAL VALUE

This benefit has an indirect causal chain: ambient documentation ' less time charting ' more time at bedside ' better assessments ' fewer adverse events. The mechanism is well-supported in literature, but direct attribution is complex. We show this separately from hard ROI so you can evaluate it with appropriate context.

YOUR CALCULATION

STEP 1: BASELINE FALL RATE

How many falls occur annually?

600

 beds x

3.0%

 rate =

216.0 falls

Average fall rate: 3-5 per 1,000 patient days.

STEP 2: PREVENTION POTENTIAL

How many could be prevented?

216.0

 falls x

10%

 prevention =

21.6 avoided

Conservative 10% through better bedside time.

STEP 3: COST AVOIDANCE

What's the potential value?

21.6

 avoided x

\$7K

 cost =

\$46K

Fall cost: \$3K-\$30K average. We use \$6,500.

Annual Falls Prevention Value

0.0 falls x \$7K

\$46K

WHAT THIS MEANS

If 21.6 falls are prevented through improved bedside presence and situational awareness, that's \$46K in avoided costs. This is potential value—attribution is indirect but the mechanism is clear.

THE FULL PICTURE

What else gets better—and why we don't put a number on it

Your ROI model captures outcomes we can measure with confidence: overtime, retention, agency spend, and (with caveats) patient safety events. But ambient documentation also improves areas that are harder to quantify—and often matter just as much to your organization.

We're showing these separately because we want to be honest about what we can and can't measure. These benefits are real. They show up in surveys, in audits, in patient feedback, in the daily experience of nurses. But attributing a dollar value would require assumptions we're not comfortable making.

Patient Experience (HCAHPS)

Patients notice when nurses are fully present versus distracted by documentation. The research is clear: nurse communication scores correlate with bedside presence. When nurses chart at the bedside on a laptop, patients perceive divided attention. When nurses are fully present—making eye contact, listening actively, explaining clearly—scores improve.

Ambient documentation removes the laptop from the interaction. The nurse can focus on the patient. The charting happens automatically.

Impacts: "Nurse listened carefully" scores, "Nurse explained things" ratings, overall nurse communication domain, VBP reimbursement tied to HCAHPS

Why we don't quantify: HCAHPS is influenced by dozens of factors—staffing, acuity, room cleanliness, food quality, pain management. Isolating the documentation effect is nearly impossible.

Survey & Compliance Readiness

Complete documentation is your first line of defense in any survey. Joint Commission, CMS, state surveys—they all start with the chart. When documentation is complete, timely, and accurate, surveyors find what they're looking for. When it's not, you get findings, corrective action plans, and follow-up visits.

Real-time documentation means charts are always current. There's no end-of-shift catch-up. No "I'll finish that later." The documentation exists because the conversation happened.

Impacts: Survey deficiency rates, time spent preparing for surveys, remediation costs after findings, staff anxiety around survey readiness

Why we don't quantify: Survey outcomes are binary and infrequent. Attributing a dollar value would require assumptions about deficiency probability that vary too much by organization.

Care Coordination

Handoffs are only as good as the documentation behind them. Shift-to-shift handoffs, department-to-department transfers, discharge planning—they all depend on complete, accurate, timely documentation. When documentation lags, information gets lost. When it's real-time, everyone works from the same current picture.

Impacts: Shift-to-shift handoff quality, interdepartmental communication, miscommunication-related safety events, time spent "hunting" for information

Why we don't quantify: Care coordination benefits are diffuse and hard to isolate. Better handoffs lead to fewer errors, faster care, better outcomes—but the causal chain is long and confounded.

Why This Matters

We could have assigned dollar values to these outcomes. Other vendors do. But we'd rather give you a conservative ROI you can defend than an inflated one that falls apart under scrutiny.

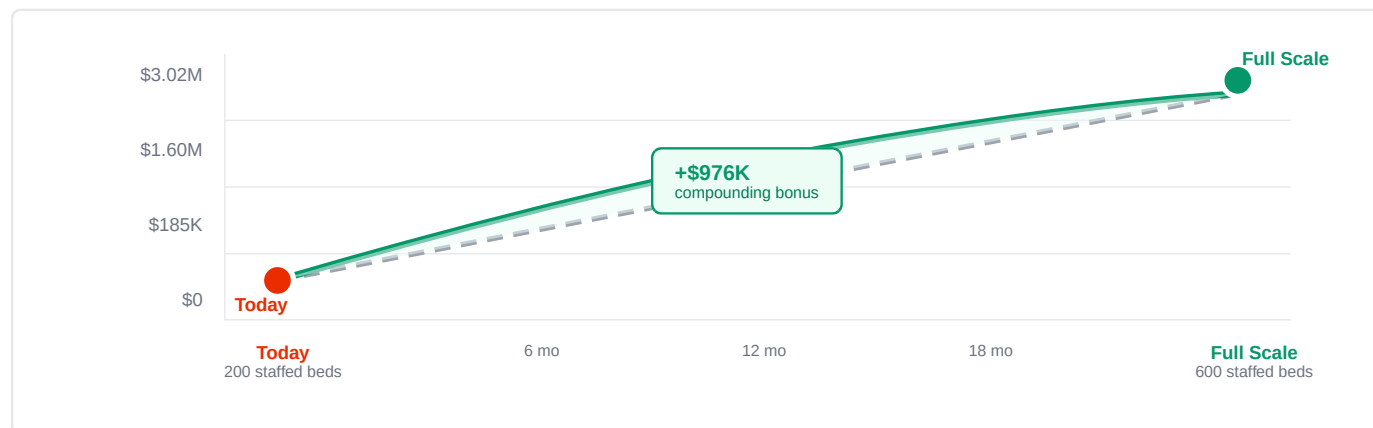
These outcomes are real—they show up in surveys, in patient feedback, in the daily experience of nurses. And they matter to your board, your CNO, and your patients.

YOUR VALUE JOURNEY

From pilot to full scale

Implementation follows a proven path. Starting with a pilot group of 200 staffed beds, you'll prove value quickly before expanding. This isn't just about adding more units—it's about building compounding returns as adoption increases.

At full scale with 600 staffed beds and 80% utilization, annual value reaches \$3.02M—a 16.4x increase from pilot. The journey matters as much as the destination.



Pilot Phase

- 200 staffed beds
- 379,600 documentation events
- 65% utilization target
- Annual value: \$185K

Full Scale

- 600 staffed beds
- 80% utilization target
- 24 months timeline
- Annual value: \$3.02M

THE COMPOUNDING EFFECT

+\$976K

Additional annual value from network effects at full scale

Value doesn't grow linearly—it compounds. As more units adopt Abridge, documentation quality improves floor by floor. Overtime patterns shift. Agency reliance drops. The value per bed increases even as you add beds.

This is why pilot success often understates full-scale potential. At 80% utilization with 600 staffed beds, network effects add \$976K to your annual value—bringing total potential to \$4.00M.

CONNECTED VALUE

Nursing + Inpatient compounds your results

This assessment models nursing-specific value. Organizations implementing Abridge across nursing and inpatient settings see compounding benefits: nurses charting faster means hospitalists get better context, CDI has cleaner records to work with, and the entire documentation ecosystem improves.

METHODOLOGY & ASSUMPTIONS

How we built your model

YOUR INPUTS

600 staffed beds
900 nurse FTEs
80% expected utilization
5 min saved per documentation event
\$150/staffed bed/month

LABOR COST BENCHMARKS

Metric	Range	Default
RN Base Hourly Rate	\$40-\$50	\$45
OT Rate (1.5x)	\$60-\$75	\$60
Agency Premium	\$65K-\$85K/FTE	\$75K/FTE
Replacement Cost	\$40K-\$60K	\$50K
Turnover Rate	18-25%	20%

QUALITY & SAFETY BENCHMARKS

Metric	Range	Default
HAPI Cost	\$10K-\$50K	\$20K
Fall Cost	\$3K-\$30K	\$6,500
HAPI Rate	2-3%	2%
Fall Rate	3-5%	3%

Conservative by Design

Every assumption here can be adjusted. We start conservative because credibility matters more than inflated projections. Your actual results may differ—often in positive ways we didn't model.

"Nursing documentation carries a different kind of weight—it's not just about coding or billing. It's about **patient safety**, **regulatory compliance**, and the daily experience of nurses who spend 25-35% of their shift charting instead of caring. The ROI is measurable. The impact on culture is harder to quantify—but often matters more."

This assessment represents a starting point. The calculations here are based on industry benchmarks and conservative assumptions. Your organization's actual experience will depend on current documentation workflows, nurse engagement, and implementation approach.

What this model doesn't capture: the relief on nurses' faces when they realize they can go home on time. The quality of patient interactions when nurses aren't typing. The cultural shift when documentation stops being a burden.

Those outcomes matter. They're just harder to put in a spreadsheet.

YOUR SUMMARY

Total Annual Value (Labor)	\$637K
Potential Value (Quality)	\$46K
Annual Investment	\$360K
Net Value Created	\$277K